



INTERPERSONAL RELATIONSHIPS IN FIRST LEVEL CARE WORKERS OF THE MINISTRY OF HEALTH, BREÑA DISTRICT, 2022

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ABSTRACT

Purpose: Determine the level of interpersonal relationships of the health worker at the first level of care in the Breña District in the year 2022.

Theoretical framework: Good treatment, which is given and received, is the way we proceed or act when relating to another human being, group or organization, whether by deed or omission, physically or verbally, etc.

Material and Methods: The present study is descriptive, retrospective and cross-sectional, in 68 health workers. The survey on interpersonal relationships by Muñoz (2015) was applied.

Results and Conclusions: Confirmatory factor analysis was applied to obtain the construct validity of the instrument, meeting the requirements: Kaiser-Meyer-Olkin (KMO) index of 0.811 (>0.7) and Bartlett test with a significance value of $0.00 < 0.05$. In the health workers of the Breña district, a high level is observed 19.1%, a medium level 77.9% and a low level 2.9%. Considering the 3 dimensions, the medium level varies between 59.4% to 80.6% and in terms of proportions, in second place stands out the high level whose values vary between 16.7% and 34.4%.

Research implications: The instrument showed evidence of construct validity and reliability, and these analyzes indicate the feasibility of the instrument proposed by Muñoz, who distributed the 18 items in three dimensions: attitude, treatment and communication, considering 6 items for each one.

Originality/value: It is important to establish adequate interpersonal relationships based on fluid communication that will contribute to the fulfillment of the institution's objectives focused on achieving of health indicators and achieve quality care.

Keywords: Interpersonal Relations, Health Workers, Health Center.

RELAÇÕES INTERPESSOAIS EM TRABALHADORES DE PRIMEIRO NÍVEL DO MINISTÉRIO DA SAÚDE, DISTRITO DE BREÑA, 2022

RESUMO

Objetivo: Determinar o nível de relações interpessoais do profissional de saúde no primeiro nível de cuidados no Distrito de Breña no ano de 2022.

Estrutura teórica: O bom tratamento, que é dado e recebido, é a maneira como procedemos ou agimos quando nos relacionamos com outro ser humano, grupo ou organização, seja por ação ou omissão, física ou verbalmente, etc.

Material e Métodos: O presente estudo é descritivo, retrospectivo e transversal, em 68 profissionais de saúde. Foi aplicado o levantamento sobre as relações interpessoais de Muñoz (2015).

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Resultados e Conclusões: A análise fatorial de confirmação foi aplicada para obter a validade de construção do instrumento, atendendo aos requisitos: índice Kaiser-Meyer-Olkin (KMO) de 0,811 ($>0,7$) e teste de Bartlett com valor de significância de $0,00 < 0,05$. Nos profissionais de saúde do distrito de Breña, observa-se um nível elevado de 19,1%, um nível médio de 77,9% e um nível baixo de 2,9%. Considerando as 3 dimensões, o nível médio varia de 59,4% a 80,6% e em termos de proporções destaca-se o alto nível cujos valores variam de 16,7% a 34,4%.

Implicações da pesquisa: O instrumento mostrou evidências da validade e confiabilidade do construto, e essas análises indicam a viabilidade do instrumento proposto por Muñoz, que distribuiu os 18 itens em três dimensões: atitude, tratamento e comunicação, considerando 6 itens para cada um.

Originalidade/valor: É importante estabelecer relações interpessoais adequadas com base em uma comunicação fluida que contribua para o cumprimento dos objetivos da instituição focados na obtenção de indicadores de saúde e na obtenção de cuidados de qualidade.

Palavras-chave: Relações Interpessoais, Profissionais de Saúde, Centro de Saúde.

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1 INTRODUCTION

The recent report from the World Health Organization considers work as one of the main social determinants of health and, according to the International Labor Organization, enjoying it is a requirement to achieve maximum well-being (Benavides, 2011). Work provides the health worker with personal benefits, but in the same way, said worker constitutes a fundamental element in the production of health services; His participation in the effective execution of health policies and in the structuring of health systems and adequate services is essential.

Work in the health area is characterized by the relationship between workers who need to act daily in different scenarios. For this reason, the work team observes, in addition to disparities in technical execution, disparities in social position and valuation, as well as differences in relation to autonomy in decisions, which can trigger inequalities in the complex world of work. that lead workers to express and prove their differences.

The first level of care, within which the Health Centers are located, directs its actions to the individual, the family and the community. For the development of these activities, working relationships are generated between the leadership, health worker and users, therefore, it is important to establish adequate interpersonal relationships based on fluid communication that will contribute to the fulfillment of the institution's objectives focused on achieving of health indicators and achieve quality care.



The Ministry of Health, for decades, has been developing strategies focused on improving health care at the first level, which are not sufficient due mainly to the lack of Human Resources in Health (RHUS). This is reflected in the estimated gap in 2012, which has been calculated at 69,904 RHUS for the First Level of Care of the Ministry of Health and Regional Governments (Buchan, 2000).

In a social network such as a Health Center, negative interactions are likely to arise based on the ties of intimacy that are intertwined. Conflictive relationships have a strong negative effect on the perception of quality of life, and even affect the level of life satisfaction and reduce self-esteem (Rosenfield and Wenzel, 1997); likewise, relational tensions have been associated with a greater risk of mental health problems in both men and women (Stansfeld, Fuhrer & Shipley, 1998; Ávila-Toscano & Madariaga, 2015). The term conflict is perceived as if they were phenomena, facts, behaviors that, in institutional life, constitute "noise" and are recognized as such by workers and management.

Health establishments are responsible for health promotion and disease prevention, not only of the population they serve, they must also prevent injuries, illnesses and disabilities and promote the health of their workers by improving working conditions as well as promotion with the aim of enjoying a healthy working life (Benavides et al., 2018). Currently, health systems prioritize terms such as efficiency, effectiveness and productivity, which causes competitive behavior and the creation of power relationships among health workers, generating feelings of frustration and discontent that will influence patient care.

The WHO and PAHO indicate that health promotion in the workplace includes the implementation of a series of policies and activities within the same institution, designed to help employers and workers at all levels to increase control over their health and improve it, promoting the productivity and competitiveness of institutions and contributing to the economic and social development of countries.

Health workers constitute a high-risk group for the development of stress-related problems. This is due to the characteristics of their work, which includes exhausting situations that they have to face during daily activities, with high demands and multiple psychosocial factors, as well as the transfer they receive from their patients. International organizations such as the World Health Organization (WHO) and Labor International (ILO) draw attention to psychosocial aspects that affect health workers, such as the framework guidelines to combat workplace violence in the workplace. Health sector; mention that although workplace violence affects all sectors of workers, one of the sectors most at risk is health care, since of the violence experienced in the workplace, the health sector occupies the fourth part, relating to tension at



work, social instability, deterioration of interpersonal relationships and other psychosocial factors in the workplace. These factors may be present in more than half of the workers and are more present in women (Da Silva et al., 2015).

It is important that health workers be sensitive to the emotions of their colleagues to adequately manage situations of interpersonal conflicts and emotional regulation, which arise from daily work situations, especially in healthcare work. Adequate emotional management can even prevent situations of workplace aggression, since in this way the spiral of aggression that usually occurs between uncivil behaviors and that can lead to situations of continuous aggression and a toxic work environment is broken.

The objective of this work is to determine the level of interpersonal relationships of the health worker at the first level of care in the Breña District, in order to obtain a series of descriptive measures of the questions asked through a survey.

2 THEORETICAL FRAMEWORK

According to Minsa in the Management and Interpersonal Relations Manual, three components are considered: communication, treatment and attitude. Satir (1980) indicates that communication means interaction or transaction and within a social context, it can be verbal and non-verbal, and includes all the symbols and cues that people use to give and receive meaning.(eleven). Non-verbal communication is widely used in the health field and corresponds to approximately 70% of the language used and allows expressing feelings and emotions, which is why it is very important that health workers have basic knowledge on the subject. It also provides the worker with the opportunity to inform and encourage changes in the behavior of the patient, family or community, with the aim of increasing knowledge about their health status and allowing them to make their own decisions.

Attitudes are relatively stable predispositions that determine behavior and patterns of social conduct. According to Shunk, they are internal beliefs that influence personal actions and reveal attributes such as generosity, honesty or lifestyles. The main elements that make up attitudes can be reduced to cognitive, affective and reactive components, which tend to be resolved in action in a certain way.

Good treatment, which is given and received, is the way we proceed or act when relating to another human being, group or organization, whether by deed or omission, physically or verbally, etc. When this relationship is more or less lasting, we then speak of coexistence; and if it occurs appropriately, we can talk about quality interpersonal relationships or peaceful



coexistence. In any case, active coexistence, as they call coexistence from complex epistemological perspectives, is understood as the true acceptance of that other (human being, group or organization), that is, its legitimation.

3 METHODS

3.1 STUDY DESIGN

The present study is descriptive, retrospective and cross-sectional; The data collected through a survey applied to professional, technical and administrative health workers who are working under the regimes were included: appointment, CAS contract and third party who are working for more than 1 year at the Chacra Colorada and Breña Health Centers, both belonging to the district of Breña.

3.2 THE INSTRUMENT TO EVALUATE INTERPERSONAL RELATIONSHIPS

For the evaluation of interpersonal relationships, the questionnaire developed by Muñoz (2015) was considered, which is divided into three dimensions: communication, attitudes and treatment(10), made up of a total of 18 items on a Licker scale, when this instrument is applied it takes an average of 30 minutes. For content validity, the author considered an expert judgment; in this research, construct validation has been considered through a confirmatory factor analysis, which was carried out to confirm the relationship between the indicators and the dimensions (B & H, 2007).

3.3 POPULATION, SAMPLE AND SAMPLING

The population is made up of 58 workers from the Chacra Colorada Health Center and 64 workers from the Breña Health Center, the sample was obtained through a stratified probabilistic sampling that reported 32 workers from the Chacra Colorada health center and 36 from the Breña health center.

3.4 STUDY VARIABLES



In the data collection survey, information was obtained on the demographic characteristics of health workers such as age, sex, marital status, length of service, work group and length of service, as well as the characteristics of interpersonal relationships within their work environment.

3.5 STATISTIC ANALYSIS

For the development of the statistics of this research, two parts are considered, the first is the descriptive, with the application of tables, graphs and summary measures appropriate to the variable analyzed, the second part considers the inferential statistics through the application of a test. of proportions, and for construct validation a confirmatory factor analysis was applied.

3.6 ETHICAL ASPECTS

The present investigationIt was executed taking into account the ethical principles: voluntary participation, beneficence, equity and confidentiality,It was approved by the management of both health centers. The information obtained from the workers was handled with a high criterion of confidentiality, for this reason a code was assigned to identify the workers so that their name does not appear during the study.

4 RESULTS

The statistical analysis confirmed that the survey had good internal consistency($\alpha = 0.771$), in which the results of the factor analysis (Table 1) consider a model with a total of 6 items per dimension. It is highlighted that the model proposed by the factor analysis presented a good fit according to the statistics evaluated ($KMO=0.811$, Bartlet test reported a significance value of $0.00 < 0.05$)

The results of the confirmatory factor analysis allow us to ratify what was proposed by Muñoz (2015), who distributed the 18 items into three dimensions:

Dimension 1: communication.- Refers to the verbal and non-verbal interaction that takes place between coworkers in a health center. This dimension is made up of the items: p01, p02, p03, p04, p05 and p06.



Dimension 2: Attitude.- Refers to the tendency to act in a certain way in the face of a certain situation generated by co-workers in a Health Center. This dimension is made up of the items: p07, p08, p09, p10, p11 and p12.

Dimension 3: treatment.- Refers to achieving and maintaining the trust, collaboration and understanding of coworkers at a Health Center. This dimension is made up of items: p13, p14, p15, p16, p17 and p18.

Table 1

Application of confirmatory factor analysis to the 2022 survey on interpersonal relationships among health workers.

Rotated component matrix				
		Component		
		1	2	3
p03	My colleagues quickly understand when I verbalize a message.	.866		
p02	When I have something to say, my partner(s) listens to me without showing discomfort even if I am in a hurry.	.845		
p06	My colleagues know how to keep quiet and listen when you need it.	.773		
p01	When I have argued with a colleague, he (she) shows a willingness to resolve the disagreement as soon as possible and without resentment.	.751		
p04	When I have a problem and I talk to a colleague about it, he (she) shows interest in the topic.	.736		
p05	When I talk to my colleagues about something that worries me, I feel comforted because they give me strength to solve the problem.	.576		
p09	It is easy for me to put myself in my partner's shoes and understand their behavior even when it makes me feel uncomfortable.		.878	
p11	When a colleague has a problem, there is little interest from others in helping them.		.757	
p08	When faced with a problem, my colleagues put themselves in my shoes and try to understand how I feel.		.693	
p12	Some colleagues make negative criticisms of others in their absence.		.600	
p07	Even when disagreements occur, respect between colleagues prevails.		.551	
p10	When a classmate makes a mistake, the others help him or her so that it doesn't happen again.		.477	
p17	My colleagues congratulate others on their birthdays and special events.			.804
p16	My colleagues act with simplicity			.796
p18	My colleagues voluntarily help another person when they are overloaded with work.			.703
p15	My colleagues, when requesting something, do so by saying "please."			.639
p13	My colleagues greet others politely when entering the establishment or service.			.527
p14	My colleagues forget to say thank you when someone has done them a favor.			-.473
Extraction method: Principal component analysis.				
Rotation method: Varimax normalization with Kaiser.				
to. The rotation has converged in 6 iterations.				

Source: Prepared by the author, (2024)

The survey was applied to 68 health workers at the first level of care in the district of Breña, with greater female participation (68%), care work personnel (63%) and appointed



personnel (88%). The predominant age range is 45 to 65 years (50%) with a minimum age of 26 years and a maximum of 70 years, the service time is greater than 10 years (81%) as shown in table 2.

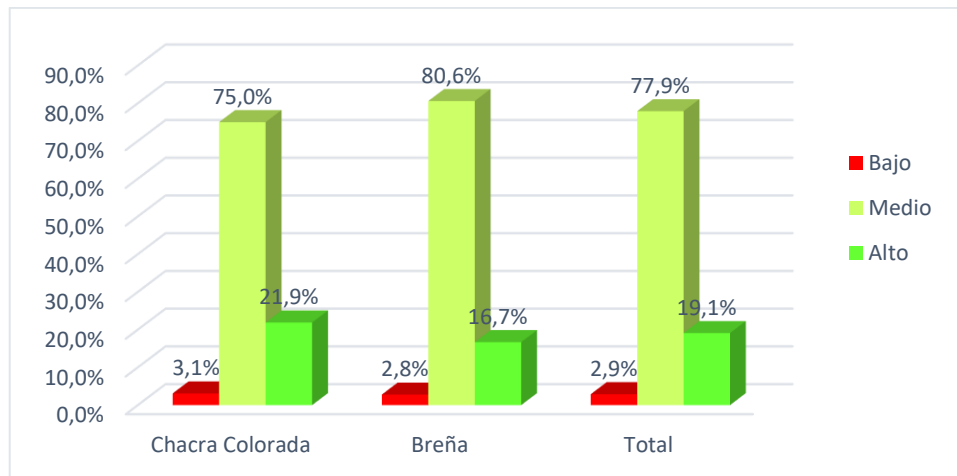
Table 2

Sociodemographic characteristics of health workers in the Breña district 2022.

		Red Chakra		Brena		Total	
		n	%	n	%	n	%
Sex							
	Male	4	12.5%	18	50.0%	22	32.35%
	Female	28	87.5%	18	50.0%	46	67.65%
Age						0	
	20-30	2	6.3%	1	2.8%	3	4.41%
	31-45	10	31.3%	13	36.1%	23	33.82%
	46-65	18	56.3%	16	44.4%	3. 4	50.00%
	>65	2	6.3%	6	16.7%	8	11.76%
Civil status							
	Single	10	31.3%	3	8.3%	13	19.12%
	Married	14	43.8%	32	88.9%	46	67.65%
	Cohabitant	6	18.8%	0	0.0%	6	8.82%
	Divorced	0	0.0%	1	2.8%	1	1.47%
	Widower	2	6.3%	0	0.0%	2	2.94%
Employment situation							
	Named	28	87.5%	32	88.9%	60	88.24%
	cas	3	9.4%	4	11.1%	7	10.29%
	Third	1	3.1%	0	0.0%	1	1.47%
Service time							
	Less than one year	0	0.0%	0	0.0%	0	0%
	1-3 years	1	3.1%	0	0.0%	1	1.47%
	4-6 years	4	12.5%	4	11.1%	8	11.76%
	6-10 years	3	9.4%	1	2.8%	4	5.88%
	More than 10 years	24	75.0%	31	86.1%	55	80.88%
Staff							
	Professional	10	31.3%	5	13.9%	fifteen	22.06%
	Assistance technician	eleven	34.4%	17	47.2%	28	41.18%
	Administrative technician	10	18.8%	10	27.8%	twenty	29.41%
	Others	1	3.1%	4	11.1%	5	7.35%

Source: Prepared by the author, (2024)

Regarding interpersonal relationships in health workers in the district of Breña, it is observed that 77.9% have a medium level, followed by 19.1% with a high level and 2.9% with a low level, in the same way when comparing the 2 Health Centers. , the proportions by levels are similar. (Figure 1)

**Figure 1***Level of interpersonal relationships in health workers in the Breña district 2022.*

Source: Prepared by the author, (2024)

Regarding the level of interpersonal relationships in health workers, according to the dimensions of communication, attitude and treatment, in both health centers the average level stands out, as seen in Table 2, the proportion of the average level. In all cases it varies between 59.4% to 80.6%, secondly the high level stands out whose values vary between 16.7% and 34.4%.

Table 2*Interpersonal relationships, according to dimension, of health workers in the district of Breña 2022.*

Dimension and/or variable	Levels	Red Chakra		Brena		Total		Statistical test
		n	%	n	%	n	%	
Communication	Low	3	9.40%	2	5.60%	5	7.40%	Zcal=1.419 Pvalue=0.156
	Half	19	59.40%	28	77.80%	47	69.10%	
	High	10	31.30%	6	16.70%	16	23.50%	
Attitude	Low	3	9.40%	1	2.80%	4	5.90%	Zcal=1.419 Pvalue=0.156
	Half	19	59.40%	29	80.60%	48	70.60%	
	High	10	31.30%	6	16.70%	16	23.50%	
Deal	Low	1	3.10%	1	2.80%	2	2.90%	Zcal=1.121 Pvalue=0.262
	Half	twenty	62.50%	27	75.00%	47	69.10%	
	High	eleven	34.40%	8	22.20%	19	27.90%	
Relationships	Low	1	3.10%	1	2.80%	2	2.90%	Zcal=0.542 Pvalue=0.588
	Half	24	75.00%	29	80.60%	53	77.90%	
	High	7	21.90%	6	16.70%	13	19.10%	
	Total	32	100.00%	36	100.00%	68	100.00%	

Source: Prepared by the author, (2024)

For the interpersonal relations variable, in the communication dimension, it was found that 31.30% of the workers at the Chacra Colorada Health Center presented a high level, a result



that exceeds that observed at the Breña Health center, where 16.70 were reported. % (Benavides et al., 2018) workers who presented a high level, in this way it can be stated that the difference between the high percentages is not significant ($P \text{ value}=0.156 >0.05$)

For the attitude dimension, it was found that 31.30% of the workers at the Chacra Colorada Health Center presented a high level, higher than that observed at the Breña Health center, where 16.70% (Benavides et al., 2018) were reported to also present a high level, therefore, it can be stated that this difference between the high percentages is not significant ($P \text{ value}=0.156 >0.05$)

In the case of the treatment dimension, it was found that 34.40% of the workers at the Chacra Colorada Health Center presented a high level, higher than that observed at the Breña Health center, where it was reported that 22.20% (Da Silva et al., 2015). I present a high level, therefore, it can be stated that this difference between the high percentages is not significant ($P \text{ value}=0.262 >0.05$)

For the interpersonal relations variable, it was found that 21.90% of the workers at the Chacra Colorada Health Center presented a high level, higher than that observed at the Breña Health center, where it was reported that 16.70% (Benavides et al., 2018) presented a high level, therefore it can be stated that this difference between the high percentages is not significant ($P \text{ value}=0.588 >0.05$).

5 DISCUSSION

It was possible to validate the survey on interpersonal relationships among health workers. The items referring to the 18 components proposed for the evaluation have sufficient construct validity and reliability for their application in other studies on interpersonal relationships.

According to the INEI, by 2023 there will be 356 health centers in metropolitan Lima. when comparing the results on job training of health workers in the district of Breña, we found that it is similar to the national reality, where healthcare personnel represent approximately 79% of workers in the health sector. In relation to gender, in the present study, women whose age range is over 60 years is the one with the highest percentage, socialization in older adults is an important factor in the diagnosis of loss of autonomy, which in Most cases are associated with causes such as retirement, widowhood, loneliness and family conflicts, among others. others(twenty), which can influence inadequate interpersonal relationships.



The highest percentage of workers have the employment status of appointed and have been working for more than 10 years.

6 CONCLUSION

Through the systematization of the results, it was found that the interpersonal relations variable in health workers is medium and high with a level of significance equal to (0.156), this coincides with other studies where such levels were also found, such as Hanco and Carpio (twenty-one) that According to the results obtained, the interpersonal relationship in the workers is good, because many times the aspects are carried out: communication, attitude and treatment, which are expressed through channels and complete communication system, where the information clearly and effectively. Regarding communication, a medium-high level is presented, which coincides with .Moreno and Pérez indicate that communication is the dimension that most correlates with interpersonal relationships for improving the performance of workers, allowing us to conclude that it has an impact directly, because maintaining good interpersonal relationships favors and increases the productive level in any institution.

Regarding attitude,

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